

COGNITIVE BEHAVIORAL THERAPY (CBT) – SESSION 2

New Case: Rohan (26 y/o) – Contamination OCD + Checking

Highly Detailed – Realistic Style

SESSION INFORMATION

Client:	Rohan (26 years, Male, Software Engineer, Bangalore)	Therapist:	Dr. A.
Session #:	2 of 18–20	Focus:	Understanding the OCD Cycle + Coping Strategies

PRE-SESSION THERAPIST NOTES

Client arrived looking slightly more open than last session but still visibly tired and somewhat ashamed. He completed the monitoring homework on 5 out of 7 days. He reported washing his hands excessively (estimated 25–35 times on most days) and checking switches/locks multiple times before bed. He noted that anxiety spikes significantly when he tries to resist a compulsion. Today's plan: Review monitoring homework in detail, help him understand his personal OCD cycle more clearly, introduce simple coping strategies (urge surfing + labeling thoughts), continue building motivation and alliance, and assign new homework. Avoid starting ERP yet.

SESSION START – HOMEWORK REVIEW

Therapist: Good morning, Rohan. How has your week been? I see you brought the monitoring notes. How was it tracking the washing and checking?

Client: (speaking in a low voice) It was difficult to write everything down. I felt like I was seeing how bad it really is. On most days I washed my hands around 25–35 times. Sometimes more if I went outside. I also checked the gas knob and switches 4–5 times every night before sleeping. I noticed that whenever I try to stop myself from washing or checking, my anxiety shoots up very fast and I feel like I can't think about anything else until I do it.

Therapist: Thank you for doing that even though it was uncomfortable. Writing it down can make the reality of how much time and energy OCD is taking feel very clear — and sometimes painful. You mentioned that when you try to resist, the anxiety increases quickly. What happens to that anxiety after you complete the compulsion? Does it go away completely?

Client: It goes down a lot, but not completely. And it doesn't last long. After 10–15 minutes, sometimes even sooner, the thought comes back again — like "What if I didn't wash properly?" or "What if I touched something without realizing?" So I end up washing or checking again. It feels like a loop I can't get out of.

UNDERSTANDING THE PERSONAL OCD CYCLE

Therapist: What you just described is the classic OCD cycle. Let me draw it out simply:

- **Obsession/Trigger**** → You touch something (or think you might have) → Thought: "I might be contaminated / something bad might happen"
- **Anxiety/Urge**** → Anxiety rises quickly. You feel strong urge to wash or check to "neutralize" the threat.
- **Compulsion**** → You wash hands excessively or check multiple times.
- **Temporary Relief**** → Anxiety decreases for a short while.
- **Cycle Repeats**** → The relief doesn't last. The obsession returns, sometimes stronger, and the cycle continues.

Therapist: Does seeing it written like this match what you experience?

Client: Yes... exactly. Especially the part where the relief doesn't last and the thought comes back. That's why I keep doing it again and again. I know it's not logical, but in the moment it feels like I have to do it or something bad will happen.

Therapist: That's a very important observation. The fact that the relief is temporary and the thoughts return is actually one of the reasons OCD stays strong. Every time you do the compulsion, your brain gets a short-term "reward" (anxiety goes down), so it learns to send the obsession again next time. Over time, this makes the OCD stronger, not weaker. This is why we eventually work on breaking the cycle by reducing compulsions — but we'll do that gradually when you're ready.

INTRODUCING SIMPLE COPING STRATEGIES

Therapist: While we prepare for ERP, there are some smaller strategies that can help you handle the urge and anxiety in the moment without fully giving in to the compulsion. One useful skill is called ****Urge Surfing****. The idea is to treat the urge like a wave — it rises, peaks, and then naturally falls if you don't act on it. Instead of immediately washing or checking, you can notice the urge, label it ("This is an OCD urge"), and ride it out for a few minutes while using breathing or grounding. Would you like to try learning this?

Client: I can try. But I'm not sure I'll be able to stop myself from washing when the urge is very strong.

Therapist: That's completely okay. We're not expecting you to resist completely right now. This is just a tool to practice noticing the urge and delaying the compulsion even by a small amount (like 1–2 minutes at first). Over time, this can help you build tolerance to the anxiety. We can start very small.

Client: Okay. I'll try to notice the urge and wait a little before washing, at least sometimes.

HOMEWORK ASSIGNMENT

Therapist: For this week:

- Continue monitoring washing and checking (frequency + situations)

2. Practice noticing the urge and trying to delay the compulsion by 1–2 minutes when possible (use breathing or grounding during that time)

3. Label the thought when it comes: e.g., “This is an OCD contamination thought” instead of engaging with it fully

Client: I'll try the delaying part on some days. I don't think I can do it every time, but I'll try when the urge is not too strong.

Therapist: That's completely fair. We go at your pace.

SESSION SUMMARY & FEEDBACK

Therapist: Today we reviewed your monitoring, mapped out your personal OCD cycle clearly, and introduced urge surfing as a small coping tool. How are you feeling about today's session?

Client: It helped to see the cycle written down. I understand better why it keeps repeating. I'm still nervous about reducing the compulsions, but the idea of starting small with delaying makes it feel a bit more manageable.

Therapist: That's completely fair. Any questions before we finish?

Client: Not really.

POST-SESSION THERAPIST NOTES

- Client completed monitoring homework partially. Showed good insight into the temporary nature of relief from compulsions.
- Urge surfing introduced as a pre-ERP coping strategy. Client showed realistic caution but willingness to try delaying compulsions in small ways.
- Client continues to experience significant shame and exhaustion related to OCD symptoms.
- Next session: Review homework (delaying attempts), continue building coping strategies, deepen motivation for ERP, begin more detailed discussion of exposure hierarchy if client shows readiness.

Disclaimer: This is a completely fictional but realistic simulation created solely for educational and AI training purposes. It does not represent any real person or therapy.